



DOB:

Gender: F

Ref#:

Hist#:

Provider Group:

Date of Service:

Date Received:

Room:

Date Reported:

FINAL SURGICAL PATHOLOGY REPORT

Diagnosis:

A. - B.) THYROID AND CENTRAL LYMPH NODES, TOTAL THYROIDECTOMY AND
EXCISION:

- PAPILLARY CARCINOMA.
- UP TO 1.8 CM, RIGHT LOBE, CLASSICAL PATTERN.
- MARGIN STATUS: POSITIVE.
- POSTERIOR MARGIN IS INVOLVED (SEE COMMENT).
- ANTERIOR MARGIN IS VERY CLOSE.
- TWO OF THREE LYMPH NODES INVOLVED BY TUMOR (2/3).
- OTHER FINDINGS:
- LEFT THYROID WITH HYPERPLASTIC NODULE (2.7 CM).
- UNREMARKABLE PORTION OF THYMUS AND PARATHYROID.

ICD-0-3
carcinoma papillary, thyroid, NOS
8260/3
Site: thyroid, NOS C73.9
pw
9/27/12

PATHOLOGIC TUMOR STAGING SUMMARY:

Histologic type and grade: papillary carcinoma, classical variant.
Primary tumor: pT1b, 1.8 cm in greatest dimension, limited to thyroid.
Regional lymph nodes: pN1a, two of three lymph nodes involved (2/3), including right peri-thyroid and central lymph nodes.
Distant metastasis: N/A.
Pathologic tumor stage: pIII.
Margin status: R1, positive posterior margin (anterior margin is very close).
Lymphatic and perineural invasion: Not identified.

COMMENT: The prior right lobe FNA diagnosis of papillary thyroid carcinoma is noted.
The findings are communicated to

Regarding margin status: The College of American Pathology notes that "tumor at the margin (i.e. capsule and/or ink) does not correlate to incomplete excision" and that "most surgeons, endocrinologists, and nuclear medicine specialists request information on margin status.....while this makes intuitive sense and it is recommended that a positive margin be mentioned in the final pathology report, meticulous studies on the effect of positive margins and outcome in a large series of

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Patient:

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patients with long-term follow-up are lacking....indeed, there are no data to-date on the prognostic value of close margins as an independent or co-variable."

Tumor Staging Information

Data derived from current specimen. Staging in accordance with or modified from AJCC Cancer Staging Handbook, 7th Ed, and CAP protocol,

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Patient:

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Procedure:

Specimen type:

Specimen integrity:

Specimen size:

Total thyroidectomy.

Thyroid.

Intact.

Right lobe 3.3 x 1.8 x 1.2, left lobe 3.3 x 1.6 x 1.3,
isthmus 1.2 x 0.7 x 0.6 cm, 11 grams total.

TUMOR FEATURES:

Tumor size:

Tumor focality:

Tumor laterality:

Histologic type:

Lymphovascular invasion:

Tumor capsule:

1.8 cm.

Focal.

Right.

Papillary carcinoma, classical variant.

Not identified.

Not identified.

LYMPH NODE:

Two of three (2/3), with positive right peri-thyroid and
central lymph nodes.

MARGIN EVALUATION:

Distance to closest margin:

Other margins:

Positive (posterior margin).

Posterior margin is involved.

Anterior margin is very close.

PATHOLOGIC TUMOR STAGING DESCRIPTORS:

Primary tumor:

Regional lymph nodes:

Distant metastasis:

Margin status:

Pathologic stage:

pT1a.

pN1a.

N/A.

R1, positive posterior margin.

pIII.

Additional pathologic findings:

Left thyroid lobe hyperplastic nodule; fragments of
unremarkable thymus and parathyroid gland present.

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Source of Specimen:

- A. Thyroid
- B. Lymph node; Central

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Clinical History/Operative Dx:

Thyroid cancer.

Gross Description:

A. Part A is labeled total thyroid, tie is on right lobe. Initially received in the fresh state for possible Oncogenotyping studies is an 11 gram total thyroidectomy specimen: the right lobe is 3.3 x 1.8 x 1.2 cm and the left lobe is 3.3 x 1.6 x 1.3 cm, and both lobes are bridged by a 1.2 x 0.7 x 0.6 cm isthmus. The surfaces are now differentially inked as follows:

Anterior half of the right lobe of thyroid is marked blue,
Anterior half of the left lobe of thyroid is marked orange,
Posterior surfaces are marked black.

The apex of the right lobe of the thyroid is marked with a long black suture. The right lobe is serially sectioned perpendicularly through the superior-inferior long axis to reveal a fairly circumscribed, variegated, dense, light gray to softened tumor (1.8 x 1.3 x 1 cm) with papillary features, positioned in the inferior half of the lobe and within 1 mm of the posterior and 1 mm of the anterior, and 2 mm of the inferior margins. A representative portion of this lesion is submitted for Oncogenotyping studies. The lesion is entirely submitted for routine histology. The remainder of the superior third of the lobe demonstrates a meaty, deep red-tan appearance, without additional discrete lesions.

The isthmus demonstrates a meaty deep red appearance. The left lobe is now serially sectioned perpendicularly through the superior-inferior long axis, to reveal a large gelatinous, yellow-gray nodule, replacing the upper two-thirds of the lobe, and measuring upwards of 2.7 x 1.5 x 1.3 cm. The remaining cut sections of the inferior aspect of the lobe demonstrates a meaty, deep red appearance, without additional discrete nodularity. The thyroid tissue is entirely submitted for microscopic evaluation.

Cassette summary:

A1-A6) right thyroid, superior to inferior,
A7) isthmus,
A8-A12) left lobe of thyroid, superior to inferior,

B. Part B is labeled central lymph node. Initially received in the fresh state is a lobular fragment of yellow-tan fatty soft tissue, 1.8 x 1.7 x 0.8 cm. Sectioning demonstrates rubbery, yellow-gray fibrofatty soft tissue with a single discrete lymph node, 0.4 x 0.3 x 0.2 cm. The lymph node candidate is submitted within a filter bag in B1 and the remainder of the fatty soft tissue is submitted in B2 for microscopic evaluation.

Microscopic Description:

A. Microscopic sections have been examined. The microscopic findings are reflected in the diagnosis rendered.

B. Microscopic sections have been examined. The microscopic findings are reflected in the diagnosis rendered.

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Criteria	Yes	No
Diagnostic Discrepancy		X
Primary Tumor Site Discrepancy		X
HIPAA Discrepancy		X
Prior Malignancy History		X
Dual/Synchronous Primary Malignancy		X
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewed/Initials	DM	Date Reviewed: 8/30/12

lw
8/30/12

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